

ONLINE TESTING CENTER | MULTIPLE CHOICE EXAM

MODULE: QUALITY, SERVICE IMPROVEMENT AND REDESIGN (QSIR) • SBAR TOOL



Candidate Instructions

- This exam paper contains **25 Single Best Answer (SBA)** multiple-choice questions.
- All content is explicitly drawn from verified clinical communication framework directives.
- Read each question stem carefully and identify the most appropriate clinical answer.
- The full standalone **Answer Key Matrix** is located at the absolute end of this document.

QUESTION 1

What does the acronym SBAR stand for?

- A) System, Background, Analysis, Response
- B) Situation, Background, Assessment, Recommendation
- C) Status, Biometrics, Action, Review
- D) Safety, Beliefs, Assessment, Requirements

QUESTION 2

Where was the SBAR communication tool originally developed?

- A) The United States military
- B) The UK National Health Service
- C) The World Health Organization
- D) Harvard Medical School

QUESTION 3

For what specific environment was SBAR originally designed?

- A) Field hospitals
- B) Aviation cockpits
- C) Nuclear submarines
- D) Naval shipyards

QUESTION 4

What is the primary purpose of using the SBAR tool in healthcare settings?

- A) To track staff attendance parameters
- B) To enable accurate information transfer and improve patient safety
- C) To calculate complex medication dosages
- D) To replace formal electronic health records

QUESTION 5

SBAR consists of standardized prompt questions divided into how many discrete sections?

- A) Three sections
- B) Four sections
- C) Five sections
- D) Six sections

QUESTION 6

How does using the SBAR framework specifically benefit the giver of clinical information?

- A) It automatically populates their electronic shift handover report
- B) It ensures they have properly formulated their thinking before communicating
- C) It permits them to bypass direct consults with senior staff members
- D) It automates diagnostic validation tasks on behalf of the nurse

QUESTION 7

How does the SBAR tool benefit the receiver of clinical information?

- A) They know what to expect and are less likely to interrupt with premature questions
- B) They are completely excused from taking active clinical notation notes
- C) It authorizes them to delegate incoming tasks to subordinate tiers instantly
- D) It automatically flags updates across their central shift roster portal

QUESTION 8

SBAR is particularly effective at breaking down communication barriers across which parameters?

- A) Geographical sectors separating hospital structures
- B) Different clinical disciplines and varying levels of staff seniority
- C) Intra-shift handovers exclusively
- D) Discussions occurring between healthcare practitioners and client families

QUESTION 9

Ensuring the reason for communication is absolute by formulating a clear 'Recommendation' is highly valuable for which group?

- A) Inexperienced staff who may feel uncomfortable advising senior colleagues
- B) Administrative or clerical tracking representatives filing intake records
- C) Patients currently preparing for operative interventions
- D) Pharmacy technicians orchestrating baseline ward supply tasks

QUESTION 10

Under which section of the SBAR framework should you state your name, role, and the specific unit you are calling from?

- A) Situation
- B) Background
- C) Assessment
- D) Recommendation

QUESTION 11

Stating the patient's name, identifiers, and the immediate acute reason for the contact belongs to which section?

- A) Recommendation
- B) Assessment
- C) Background
- D) Situation

QUESTION 12

In the official guidelines' clinical workflow example, telling a colleague that a patient has "become suddenly short of breath" maps to which layer?

- A) Background
- B) Situation
- C) Assessment
- D) Recommendation

QUESTION 13

Where should a patient's immediate vital signs and quantitative concerns (e.g., "oxygen saturation has dropped to 88%") be explicitly introduced?

- A) Recommendation
- B) Background
- C) Situation
- D) Assessment

QUESTION 14

Which section requires you to systematically articulate the patient's primary reason for admission alongside their historical admitting diagnosis?

- A) Situation
- B) Background
- C) Assessment
- D) Recommendation

QUESTION 15

Information regarding a patient's historical medical timeline, current daily prescription therapies, and known allergy indicators belongs under which segment?

- A) Situation
- B) Assessment
- C) Recommendation
- D) Background

QUESTION 16

To properly and adequately prepare the 'Background' information layer of an SBAR briefing, what action must a nurse execute first?

- A) Initiate telephone check-ins with the patient's family members
- B) Collect and review core context directly from the patient's chart, logs, and clinical notes
- C) Order a comprehensive suite of urgent laboratory stat screens
- D) Acquire a peer's descriptive impression regarding the care unit track

QUESTION 17

Once an organ or care team initiates active SBAR integration, what immediate next task is directed by the Quality Improvement blueprint?

- A) Suspend the process intermittently to track baseline regression curves
- B) Monitor its progress to see how well it is being used and if communication has improved
- C) Generate a secondary organizational acronym framework to diversify reporting options
- D) Draft an immediate scientific journal publication based on entry data points

QUESTION 18

What is the ultimate, explicit cultural target when embedding the SBAR toolkit within an active healthcare structure?

- A) To establish it as a mandatory tracking system for overtime calculation audits
- B) For it to become 'the way things are done around here' as an intuitive everyday habit
- C) To eliminate face-to-face handover opportunities between nursing team tiers entirely
- D) To contract the direct headcount parameters requested across a typical floor schedule

QUESTION 19

Which of the following is highlighted as a highly practical visual cue or prompt to accelerate localized SBAR implementation?

- A) Enforcing formal assessment screenings at the end of each operational week
- B) Applying structural stickers on physical telephone sets, letter templates, and patient notes
- C) Positioning warning notification banners across communal staff breakout environments
- D) Enacting unique badge identifier color alterations to distinguish certified teams

QUESTION 20

What constitutes a valid, supportive peer-to-peer prompt designed to reinforce the shared SBAR structure constructively?

- A) "Why did you fail to log these parameters into the central profile earlier?"
- B) "Can I make sure I understand you? What is your recommendation here?"
- C) "Please accelerate this summary transition, as my active shift allocation window is closing."
- D) "Can you repeat the timeline sequence from the beginning?"

QUESTION 21

Why does the system-wide integration of SBAR demonstrably compress the necessity for information repetition?

- A) Because it shares a concise and focused structure
- B) Because verbal interactions are digitally cataloged via internal audio nodes automatically
- C) Because it restricts receivers from interjecting until the brief concludes entirely
- D) Because its application is restricted to non-critical care tracking channels exclusively

QUESTION 22

The SBAR tool directly guarantees that active clinical staff are sharing data points that are:

- A) Highly elongated, expansive, and textually complete
- B) Concise and focused
- C) Flexible, open-ended, and adaptive to subjective parameters
- D) Impressionistic, qualitative, and grounded in standard professional intuitions

QUESTION 23

How is the SBAR framework fundamentally defined within the clinical redesign curriculum?

- A) An advanced biological bedside diagnostic algorithm
- B) A structured form of communication
- C) A legally binding prescription administration grid
- D) An administrative long-term care facility log architecture

QUESTION 24

How does the shared structural design of SBAR assist clinicians in processing incoming complex details?

- A) It allows them to extrapolate accurate guesses regarding missing laboratory metrics
- B) It helps staff anticipate the information needed by colleagues and encourages assessment skills
- C) It mathematically maps out the clinical deterioration velocity curve curves
- D) It automatically designs the active multi-disciplinary prescription path profile

QUESTION 25

Why does the SBAR framework explicitly discourage the receiver from interrupting with premature inquiries?

- A) Because the receiver knows that those questions will likely be answered later in the structured format
- B) Because conversational interruptions trigger formal compliance penalties
- C) Because the presenter is reading from an unalterable script template
- D) Because the protocol requires interactions to manifest via electronic communication logs only

OFFICIAL EXAM ANSWER KEY MATRIX

Verify all candidate choice selections against this matrix block.

Q1	Q2	Q3	Q4	Q5	Q6	Q7	Q8	Q9	Q10
B	A	C	B	B	B	A	B	A	A

Q11	Q12	Q13	Q14	Q15	Q16	Q17	Q18	Q19	Q20
D	B	D	B	D	B	B	B	B	B

Q21	Q22	Q23	Q24	Q25
A	B	B	B	A